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AU PAIR & ELDERLY CARE GIVING

A Comprehensive Course

2023-2024



MODULE FIVE: UNDERSTANDING VARIOUS ILLNESSES & COMPLICATIONS.

After completing this module, you will be able to:

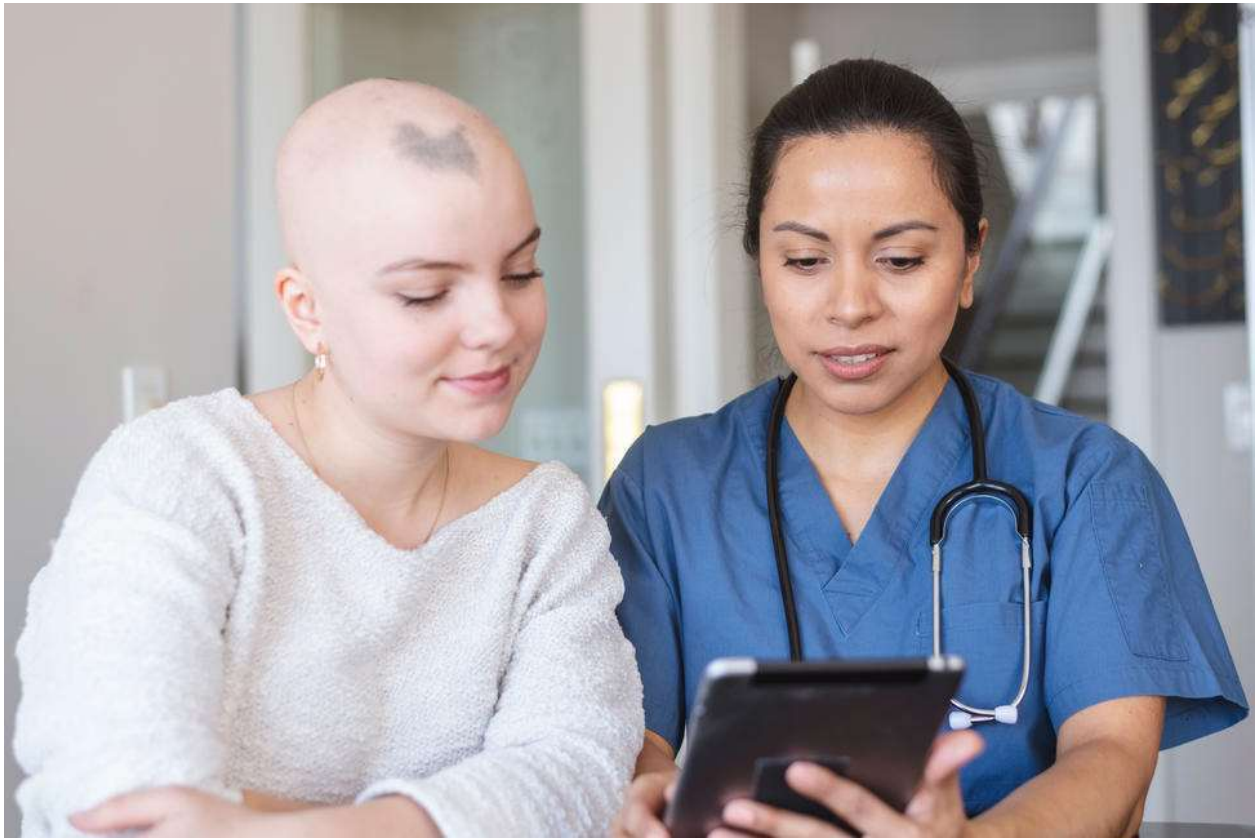
- **Know the different types of chronic conditions.**
- **Understand terminal cancer and managing its symptoms.**
- **Take care of HIV/AIDS patients and manage side effects of treatments.**
- **Handle heart disease and stroke patients.**
- **Take care of patients with diabetes and administer insulin.**



CHRONIC ILLNESSES.

One of the greatest challenges that will face health systems globally in the twenty-first century will be the increasing burden of chronic diseases (WHO 2002). Greater longevity, “modernization” of lifestyles, with increasing exposure to many chronic disease risk factors, and the growing ability to intervene to keep people alive who previously would have died have combined to change the burden of diseases confronting health systems. Chronic conditions are defined by the **World Health Organization (WHO)** as requiring “ongoing management over a period of years or decades” and cover a wide range of health problems that go beyond the conventional definition of chronic illness, such as **heart disease, diabetes and asthma**. They include some **communicable diseases**, such as the **human immunodeficiency virus** and the **acquired immunodeficiency syndrome (HIV/AIDS)**, that have been transformed by advances in medical science from rapidly progressive fatal conditions into controllable health problems, allowing those affected to live with them for many years.

They also extend to certain **mental disorders** such as **depression** and **schizophrenia**, to **defined disabilities and impairments not defined as diseases**, such as **blindness** and **musculoskeletal disorders (WHO 2002)**, and to **cancer**. While others have offered different definitions for chronic illness (*Conrad and Shortell 1996; Unwin et al. 2004*), the common theme is that these conditions require a complex response over an extended time period that involves coordinated inputs from a wide range of health professionals and access to essential medicines and monitoring systems, all of which need to be optimally embedded within a system that promotes patient empowerment.



Cancer is a group of many related diseases. All forms of cancer involve **out of control growth and spread of abnormal cells**. Normal body cells grow, divide, and die in an orderly fashion. During the early years of a person's life, normal cells divide more rapidly until the person becomes an adult. After that, normal cells of most tissues divide only to replace worn-out or dying cells and to repair injuries. ***Cancer cells, however, continue to grow and divide and can spread to other parts of the body.*** These cells accumulate and **form tumors (lumps)** that ***compress, invade, and destroy normal tissue.*** If cells break away from such a tumor, they can travel through the bloodstream or the lymph system to other areas of the body. There, they may settle and form "colony" tumors.

In their new location, the cancer cells continue growing. ***The spread of a tumor to a new site is called metastasis.*** **Leukemia**, a form of cancer, does not usually form a tumor. Instead, these

cancer cells involve the blood and blood-forming organs (bone marrow, lymphatic system, and spleen), and circulate through other tissues where they can accumulate.

Most cancer falls into five major groups:

- **Carcinoma** is a cancer that develops from cells that cover the surface of the body (skin), glands (breast, prostate), and internal organs (lungs, stomach, and intestine). Eighty to ninety percent of all cancers fall into this category.
- **Sarcoma** is a cancer that occurs in connective tissues such as bones, tendons, and cartilage, fat and muscle.
- **Leukemia** is a cancer that develops from cells in bone marrow and the lymph system that make blood.
- **Lymphoma** is a cancer that develops from cells in the glands of the immune system. Hodgkin's disease is in this group.
- **Myeloma** is a cancer that develops in the plasma cells of bone marrow.

Staging

Staging is the process of finding out how far the cancer has spread. This is very important because the choice of treatment and the outlook of recovery depend on the stage of the cancer. There is more than one system for staging. The **TNM system** is the one used most often. It gives three key pieces of information:

T refers to the size of the Tumor, measured in centimeters.

N describes how far the cancer has spread to nearby lymph nodes.

M shows whether the cancer has spread to other organs of the body.

Letters or numbers after the T, N, and M give more details about each of these factors. To make this information somewhat clearer, the **TNM** descriptions can be grouped together into a simpler set of stages, labeled stage 0 through stage IV (0-4). In general, the lower the

number, the less the cancer has spread. A higher number, such as stage IV (4), means a more serious cancer.

Types of Treatment



The four major types of treatment for cancer are surgery, radiation, chemotherapy, and biologic therapies. New kinds of treatment also available include hormone therapy (tamoxifen), and bone marrow transplants.

- **Surgery** is often the first treatment of choice for many cancers. About 60% of all people with cancer undergo surgery. If the cancer appears localized, surgery may be used to remove it along with any surrounding tissue that might contain cancer cells. Sometimes it is difficult to tell how much surgery will be needed until the surgeon sees the extent of the cancer during the operation. Success of surgery depends on whether or not the tumor has spread to other areas. About 30% of all people with cancer are cured by surgery alone. Other treatments such as radiation therapy and chemotherapy can be used along with surgery.

- **Radiation Therapy**, like surgery, is used for localized cancers. Radiation therapy is used to destroy, shrink, or damage cancer cells so much that they cannot multiply. It can be used alone, or in addition to surgery or chemotherapy. More than half of all people with cancer have radiation treatment at some point. Radiation therapy is given either through external high-energy rays, or through internal implants. External radiation is painless and is usually given in an outpatient setting. Treatment typically lasts five days a week for five to eight weeks, depending on the size, location, and type of cancer being treated. In some circumstances, radiation may be given through implants. These are small containers of radiation placed in or near the tumor while a person is under general or local anesthesia.
- **Chemotherapy** is used to treat cancer cells that have spread or metastasized to other parts of the body. Depending on the type of cancer and its stage of development, chemotherapy can be used to cure cancer, to keep the cancer from spreading, to slow the cancer growth, to kill cancer cells that may have spread to other parts of the body, or to relieve symptoms caused by cancer. Chemotherapy is treatment with powerful anticancer drugs that are most commonly given by injection into a blood vessel, or by mouth. The drugs travel throughout the body in the bloodstream. Often, a combination of anticancer drugs is used because that has proven more effective than relying on a single drug. Chemotherapy is given in cycles; each followed by a recovery period. Then another cycle of treatment is followed by a recovery period, and so on. The total course of chemotherapy is often about six months, usually ranging from three to nine months. •
- **Biologic Therapies** is a promising new treatment for certain cancers. It is sometimes called *immunotherapy*, *biotherapy*, or *biological response modifier therapy*. Biologic therapies use the body's immune system to fight cancer or lessen the side effects of some cancer treatments. Biologic therapies can act in several ways in cancer treatment. These include interfering with cancer cell growth, acting indirectly to help healthy cells, particularly immune cells, control cancer, and helping to repair normal cells damaged by other forms of cancer treatment. There are several kinds of biologic therapy now in use. You may hear terms such as *interferons*, *interleukins*, or

tumor necrosis factor. More than one kind may be used, or biologic therapy may be combined with chemotherapy or radiation therapy.

- **Complementary Therapy** refers to supportive methods that are used to enhance mainstream treatments. Examples include *relaxation training*, *acupuncture*, and biofeedback. Complementary methods do not cure disease; rather they help control symptoms and improve well-being.
- **Alternative therapy** refers to treatments that have not proven to be effective in treating a particular disease and are taken instead of conventional treatment. Examples of these include *hydrogen peroxide therapy*, *hydrazine sulfate*, and *essiac tea*.

Side Effects of Treatment

Side effects of chemotherapy and radiation therapy depend on the part of the body being treated, the type of drugs used, the amount taken, and the length of treatment period. The most common side effects are *nausea* and *vomiting*, *weight loss*, *temporary hair loss*, *increased chance of infections and fatigue*. Some side effects may be harder to cope with, while others may be mild. Fortunately, most side effects can be controlled with medications, supportive care measures, or by changing the treatment schedule.

Personal Care for a Cancer Patient

1. Caring for the Skin

Skin problems are not usually an emergency, but can be upsetting and uncomfortable for the person who has them and can indicate other medical problems. As the caregiver, you can help by noticing early signs of skin problems, by helping to treat them, and by encouraging the person with cancer to care for the skin in order to avoid future problems.

Suggestions to relieve itching and drying:

- Suggest bathing with cool water and using gentle soap
- Add baking soda to bath water
- Pat skin dry
- Limit bathing to once a day or less

- Suggest using skin lotions to help keep skin moist
- Remind the person with cancer to rinse skin thoroughly and pat dry
- Change bed sheets daily •
- Wash sheets and towels in gentle laundry soap
- Avoid extreme heat, cold, or wind
- Avoid colognes, after-shaves, or after bath splashes that contain alcohol.

2. **Hair Loss**

Some chemotherapy treatments can cause partial or complete hair loss. This may start as early as seven to 14 days after treatment begins, and hair may not grow back until six to 12 months after chemotherapy treatments are completed. Radiation to the head may cause permanent hair loss. Suggestions for caring for the scalp and coping with hair loss Encourage the person losing hair to:

- Consider getting a shorter hair style before treatments begin.
- Gently brush and wash away hair that is falling out.
- Gently clean hair and scalp with a mild protein shampoo twice a week.
- Use a satin pillow or hair net while sleeping.
- Consider cutting or shaving the last few remaining hairs.

During treatment

- Protect the head and newly exposed skin from the sun with a hat and sunblock of SPF 15 or higher.
- Wear a hat or a head scarf to retain heat in cold weather.
- Wear sunglasses to protect the eyelashes

As hair grows back.

- Use a protein conditioner.
- Avoid hair care product that contain bleach, peroxide, ammonia, alcohol, or lacquer.
- Avoid heat, curling irons, hot rollers, or blow dryers as much as possible.
- Postpone a perm on new hair.
- Keep hair short and easy to style.

3. Nausea & Vomiting

Some people never experience any nausea or vomiting from cancer, its treatments, or other medicines. Others may deal with one or both symptoms at different times in their illness, depending on which treatments they receive, and how they react to them.

Suggestions for reducing nausea and vomiting:

- Encourage eating three to four hours before treatment.
- Offer chewing gum, hard candy, or candied ginger.
- Let fresh air into the house or have the person go outside.
- Encourage rest.
- Offer sips of fluids two hours after vomiting.
- Offer dry crackers.
- Avoid unpleasant or strong odors.
- Suggest wearing loosely fitting clothes.

4. Fatigue and Tiredness

People receiving cancer treatments often may feel very worn out and tired. Tiredness may be caused by the disease itself or by the treatments. It may also be caused by anemia, which means there are fewer red blood cells circulating oxygen to the body. Anemia can be caused by cancer, its treatment, or nutritional problems. Other causes of fatigue are malnutrition or a temporary increase in waste products as cancer cells are destroyed by cancer treatments. Suggestions for promoting rest and sleep:

- Keep as active as possible during the day.
- Include an exercise plan that is approved by the doctor.
- Play relaxing music before sleep.
- Drink warm milk at bedtime.
- Take a warm bath or have a back rub at bedtime.
- Ask the doctor about sleeping pills.

Suggestions for helping the person conserve energy:

- Plan the day's activities.

- Encourage resting.
- Encourage doing things only for short periods of time.
- Encourage getting up or moving slowly to avoid dizziness.
- Serve a balanced diet with adequate protein.
- Serve snacks as well as regular meals.

5. Cancer Pain

As the caregiver, you will most likely be the person who gives pain medicines and who makes sure the health professionals are informed about and responsive to the cancer patient's pain.

When symptoms are not an emergency, but should be reported

- No relief after taking pain medicine three times, as prescribed.
- Some pain relief, but there is still significant pain after one or two days.
- A new type of pain, pain in new locations. Or new pain when moving or sitting.
- Numbness, tingling, or burning sensations that are new.
- Medicines for “breakthrough pain” are used three to four times a day in addition to their regular pain medicines.
- Tremors or involuntary jerking motions while awake or asleep

Make the best use of medicines

- Give the pain medicine at regular times, as prescribed by the doctor.
- Give the pain medicine before the pain builds up.
- Continue to give the pain medicine during the night.
- Do not stop the pain medicine suddenly if it has been taken for a number of weeks.

Ask the doctor about changing pain prescriptions, times, and doses

- Ask about increasing the amount of medicine.
- Ask about shortening the time between doses of pain medicine.
- Ask about taking short-acting or immediate-release opioids for “breakthrough pain.
- Ask about skin patches.
- Ask about time-release pills.
- Ask about subcutaneous needles attached under the skin

Manage the more common side effects of pain medicine

- Prevent constipation with diet, liquids, stool softeners, and laxatives.
- Serve food with high fiber content.
- Avoid foods that tend to cause constipation.
- Relieve a dry mouth Relieve painful dry nasal passages.
- Avoid an upset stomach.
- Relieve mild itching.
- Expect drowsiness for a few days when pain medicine is started or increased.

Managing Pain Medications

- Set an alarm as a reminder to administer pain medicine.
- Use a medicine tray or pillbox with slots for time of day to hold the medicines.
- Telephone the pharmacy before going to fill the pain prescription.
- Use the same pharmacy, if possible.
- Keep at least a five-day supply of pain medicine on hand.

Helping to Control the Pain

- Use warm showers, baths, hot water bottles, or warm washcloths.
- Use cool cloths or ice.
- Position the person carefully with pillows and soft seat cushions.
- Massage sore spots, such as the neck and shoulders.
- Encourage the person to avoid lifting or straining.
- Encourage him or her to use deep breathing exercises.
- Keep a diary, rate the pain and keep notes on what makes it worse or better.